Paediatric Haematology/Oncology Unit
Royal Hospital for Children, Glasgow

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Department of Haematology/Oncology (Schiehallion)
Royal Hospital for Children
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CYP Haematology Oncology Booster Immunisation Request / Referral for Patients Treated with Standard Dose Chemotherapy

Date: _____

PATIENT'S NAME / CHI / ADDRESS

To Whom It May Concern

This patient (birth to young adult age group) has completed standard chemotherapy and has been off all immunosuppressive therapy for a minimum of 3 months.

I would be grateful if you could arrange to have the patient re-immunised with the vaccines listed in the table below. This timetable is for children and young adults who have already completed their primary vaccination schedule prior to treatment, if they have not, there are guidance notes below the table.

This does not address patients who have received a haemopoietic stem cell transplant (HSCT), there is separate guidance for them via the HSCT team, RHC, GGC.

It has been taken from the UK Children's Cancer and Leukaemia Group's guidelines for "Vaccinations for Paediatric Patients Treated with Standard-Dose Chemotherapy" published in April 2026 and is correct at the time of releasing this SOP (Date)

CCLG members please consult www.cclg.org.uk at least annually, to ensure that no updated guidance has been released (members only).

Yours faithfully

PRINT NAME: _____

Contact details Tel. & E-mail _____

NMC/ GMC number: _____

cc Parents of (add Patient Name)
cc GP
cc (GGC Patients) Susan Hurley

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Vaccines Due from 3 months after completion of Treatment	
Name:	
CHI:	

Pathogen protected against	**Vaccine**	**Trade Name** (Equivalent alternative may be used)
Seasonal Influenza (SIIV during first 3 months after completion of treatment)	LAIV	Various
SARS-CoV-2***	SARS-CoV-2 vaccine (per national guidance)	Various
Diphtheria, tetanus, pertussis, polio, *Haemophilus influenzae* b, Hepatitis B	DTaP/IPV/Hib/HepB	Infanrix hexa or Vaxelis
Meningococcal B	Men B	Bexsero
Streptococcus pneumoniae	PCV20 (or PCV13)	Prevenar20 (or Prevenar13)
Meningococcal ACWY	Men ACWY-conjugate	Nimenrix or Menveo
Measles, Mumps, Rubella, Varicella	MMRV*	ProQuad or Priorix-Tetra
Human Papillomavirus	HPV-9**	Gardasil 9

Vaccines Names : DTaP = Diphtheria/ Tetanus/ acellular Pertussis, Hib = H. influenzae b conjugate, HepB =Hepatitis B, HPV-9 = 9-valent Human papillomavirus vaccine, IPV = Inactivated polio virus, LAIV = Live attenuated influenza vaccine, Men B = Meningococcal B, Men ACWY = Meningococcal ACWY conjugate, MMRV = Measles/Mumps/Rubella/Varicella, PCV20 = 20-valent Pneumococcal conjugate, PCV13 = 13-valent Pneumococcal conjugate, SIIV = Seasonal inactivated influenza vaccine

Further Information / Guidance

If primary vaccination schedule was not completed pre-treatment the patient should start afresh and receive the complete primary schedule.

* If the primary vaccine schedule was complete aside from the MMR then:

If no MMR vaccine prior to diagnosis, then give two doses of MMRV vaccine. If received one dose of MMR vaccine prior to diagnosis, then give two doses of MMRV vaccine. If completed two dose MMR schedule prior to diagnosis, (then 1 x MMR and 2 x V component required) give two doses MMRV.

Minimum four weeks interval between MMRV doses.

** HPV-9 vaccine should be offered to girls and boys ≥12 years old: For HPV vaccine naïve, two doses of HPV vaccine should be given at 0 and 4-6 months from starting vaccination. For girls and boys that completed the course prior to diagnosis, a booster dose should be given.

If the named vaccine is not available, an alternative with the same composition may be used.

Multiple inactivated and live vaccines can be co-administered preferably at different sites (in a different limb), however, they may be given over several appointments as per Green Book guidance on spacing intervals (at the discretion of the local immunisation team).

***SARS-CoV-2 vaccine may be given as per national guidance. For more information, please look at the full vaccination guideline on the CCLG website - https://www.cclg.org.uk

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Give booster immunisations no sooner than 3 months after completion of chemotherapy treatment.

Prevenar® (1 dose if child previously vaccinated with **Prevenar®** **OR** 2 doses - 2 months apart if no previous vaccination of **Prevenar®**)

Some of the above vaccines can be administered at the same visit at different sites and the child recalled as soon as possible for the remaining vaccines e.g. the following week or month after a live vaccine e.g. MMRV. If the family wish a solo MMR and/or solo varicella this is acceptable (if possible)

Subsequent routine booster doses (e.g. pre-school) will **not** be necessary if the child is scheduled for these within the next 12 months.

When the child is due a school leaving booster of Revaxis® ensure there is a 5 year interval between receiving the above Infanrix hexa® and the school leaving booster.

Inactivated influenza – every autumn for as long as the child is considered at risk from influenza virus infection. The intranasal live-attenuated influenza vaccine should not be used in patients or their household contacts. Note that the immune response to influenza vaccine is not optimal during the first 6 months after therapy, which is the period of greatest risk; therefore inactivated influenza vaccination should be offered to family members/household contacts.

BCG – avoid unless there is a clear case of need. **Please contact the patient's haematologist / immunologist before proceeding.**